NCE EXAM PREP

Practice Exam 13

Domain 1

Professional Development & Counselor Self-Awareness

25 Practice Questions | 33 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 03. Use the suggested 33-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> A prelicensed counselor has frozen during suicide assessments and omitted important follow-up questions in two sessions. Worried that disclosure will affect an evaluation, the counselor considers seeking advice from an outside consultant without informing the clinical supervisor.

Which professional development response is most appropriate?

A) Use only the outside consultant so the counselor can correct the problem before the next evaluation

B) Continue practicing independently and disclose the omissions only if a client later requires emergency care

C) Bring the performance concern to the clinical supervisor and develop a monitored improvement plan, using consultation only as a supplement

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Question 2

> A clinic is considering generative artificial intelligence to translate psychoeducational and consent materials into several languages. Staff members have not studied the reliability or professional implications of machine-generated clinical translations.

Which learning step should occur before the clinic relies on the translated materials?

A) Obtain independent education on translation errors, cultural and linguistic bias, confidentiality, human review, and appropriate disclosure

B) Learn how to shorten prompts because concise instructions consistently produce accurate clinical translations

C) Use the translations immediately and ask bilingual clients to report any errors they notice

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Question 3

> An active client is president of a nonprofit and invites the counselor to join its board. Board members evaluate proposals the client submits, and the counselor is drawn to the prestige and community influence the position would offer.

How should the counselor respond?

A) Accept the invitation but abstain only when the client's proposals come to a vote

B) Examine the attraction to the role, obtain consultation, and decline the overlapping position while counseling is active

C) Accept the position after telling the client that board discussions will remain separate from counseling

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Question 4

Which statement best distinguishes reflective practice from unproductive rumination?

A) Reflective practice eliminates uncomfortable emotion, whereas rumination always increases it

B) Reflective practice examines experience in a structured way to inform future action, whereas rumination repeats distress without useful movement

C) Reflective practice is completed alone, whereas rumination occurs only when discussing concerns with others

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Question 5

> A bilingual counselor wants to translate an English depression scale for Somali-speaking clients. The counselor is clinically fluent in both languages but has no training in test translation, cultural adaptation, or validation.

What is the most appropriate assessment decision?

A) Translate the items literally because language fluency preserves the scale's psychometric properties

B) Translate the scale and compare scores only among clients seen at the counselor's agency

C) Locate a supported version or consult qualified assessment and cultural-language experts before using translated scores

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Question 6

Which approach provides the strongest foundation for a counseling advocacy initiative?

A) Announce the counselor's preferred solution before gathering information so decision-makers hear a clear message

B) Engage affected people, define the barrier with relevant data, and identify feasible targets and partners for change

C) Begin public criticism of the responsible organization before checking whether the reported barrier is widespread

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Question 7

Which statement best describes professional mentoring in counselor development?

A) Mentoring can support career growth and professional identity but does not replace required clinical supervision or establish competence by itself

B) Mentoring transfers responsibility for the counselor's clinical decisions to a more experienced colleague

C) Mentoring is equivalent to supervision whenever the mentor and counselor discuss client work

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Question 8

> A school district plans to assign counselors primarily to test scheduling and routine disciplinary monitoring. Counselors have data showing that their counseling and prevention activities are associated with improved attendance and service access.

What is the most appropriate professional advocacy response?

A) Refuse all testing and disciplinary duties before discussing the plan with district leaders

B) Use role standards, local outcome data, and stakeholder input to propose duties aligned with students' counseling needs

C) Accept the reassignment because administrators alone determine the appropriate professional role of school counselors

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Question 9

> Soon after starting a new prescription medication, a counselor notices slowed thinking and two episodes of losing track of a client's narrative. The counselor is unsure whether the effects will resolve and has several complex sessions scheduled.

What is the best response?

A) Continue the schedule because medication effects are personal health information rather than a professional concern

B) Tell each client about the medication so clients can decide whether the counselor appears impaired

C) Promptly assess fitness with appropriate medical and supervisory support and arrange coverage or workload changes when needed

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Question 10

Which practice best supports a counselor in maintaining current knowledge of professional ethics?

A) Rely on the ethical principles learned in graduate school unless an employer changes a policy

B) Review informal colleague opinions because ethical standards are primarily established by local custom

C) Review current authoritative standards and guidance, use case-based education, and seek consultation on emerging questions

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Question 11

A training program labels an intervention "culturally adapted" but provides little information about the population studied or community involvement. What should a counselor do?

A) Examine the adaptation process, evidence, population limits, and relevant community perspectives before generalizing its use

B) Adopt the intervention because the cultural label indicates that it is appropriate across diverse groups

C) Reject the intervention because cultural adaptations cannot be evaluated with professional evidence

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Question 12

> A private practitioner notices reluctance to discuss termination with several clients who have met their goals. The counselor is worried about a recent drop in referrals and recognizes that continued fees would stabilize the practice's income.

What is the most appropriate response?

A) Continue sessions until referrals increase because financial stability helps all clients receive consistent care

B) Ask the clients whether they want to continue without discussing the counselor's financial concern

C) Examine the conflict in consultation, evaluate ongoing clinical need, and make transition decisions based on client welfare

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Question 13

> Minutes before a complex intake, a counselor learns that a close family member has been hospitalized. The counselor is shaking, cannot retain information from the intake form, and is uncertain about being able to focus.

What should the counselor do?

A) Assess current ability promptly and arrange appropriate coverage or rescheduling rather than beginning while significantly distracted

B) Conduct the intake as planned because personal events should never affect professional availability

C) Begin by telling the new client the full situation so the client can decide whether to continue

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Question 14

> A prospective client with severe agoraphobia needs home-based counseling and does not have stable internet access. The counselor provides only office and video sessions, while a local mobile behavioral health team has current openings.

Which response best addresses counselor-client fit?

A) Accept the client for office counseling and make attendance the first treatment goal

B) Offer video counseling because the client can locate public internet when motivation improves

C) Explain the service limitation, coordinate connection with the mobile team, and provide an appropriate bridge if feasible

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Question 15

A counselor has facilitated several bereavement groups in succession. The counselor now avoids calls from friends who are grieving, feels guilty when enjoying family events, and has difficulty remaining emotionally present during group members' stories.

What is the best response to these changes?

A) Evaluate the impact with supervision, use appropriate personal support, and consider rotating duties or reducing cumulative grief exposure

B) Stop all contact with grieving people outside work so emotional energy remains available for clients

C) Leave bereavement counseling permanently because reduced emotional presence cannot be restored

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Question 16

A counselor has a severe dog allergy that cannot be adequately managed in the available office. A prospective client requires a trained service dog during sessions. Another qualified counselor in an accessible suite can accept the client immediately.

Which fit decision is most appropriate?

A) Ask the client to attend without the service dog because counseling sessions are brief

B) Explain the environmental limitation and facilitate care with the available qualified counselor

C) Accept the client and attempt sessions until an allergic reaction actually interrupts treatment

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Question 17

A community clinic finds that hourly workers discontinue counseling more often than salaried clients. Interviews indicate that unpaid time away from work and limited appointment hours are major barriers.

Which professional growth and advocacy response is best?

A) Teach hourly workers time-management strategies before changing clinic operations

B) Maintain the schedule because offering identical hours to everyone is the fairest policy

C) Learn about economic access barriers and use client data to advocate for feasible scheduling or service alternatives

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Question 18

> An agency proposes using a brief symptom screener score as the sole rule for discharging clients. A counselor is trained to administer the screener but knows that it was designed to support, not replace, broader clinical evaluation.

How should the counselor respond?

A) Follow the rule because standardized scores are less biased than counselor judgment

B) Alter borderline scores when continued counseling appears beneficial

C) Explain the instrument's limits, advocate for appropriate interpretation, and seek assessment consultation if needed

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Question 19

> A counselor notices growing physical attraction to a client and has started dressing differently on session days and reviewing the client's messages more often than clinically necessary. No boundary violation has yet occurred.

What should the counselor do?

A) Seek supervision promptly, reinforce professional boundaries, and arrange a careful transfer if objectivity cannot be maintained

B) Tell the client about the attraction so both parties can monitor its effect on counseling

C) Ignore the reaction unless the client expresses attraction or a boundary violation occurs

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Question 20

> A counselor begins outreach to seasonal farmworkers and initially interprets missed appointments as low commitment. A community liaison explains that unpredictable work demands, transportation, and fear of lost wages shape attendance. The counselor has little knowledge of agricultural labor conditions.

Which professional development action is most appropriate?

A) Teach clients that regular attendance is necessary before adapting any services

B) Learn from qualified community and professional sources, examine the counselor's assumptions, and help the agency design feasible access options

C) Refer farmworkers to counselors from the same occupational background because training cannot address the experience gap

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Question 21

> A counselor leads a court-mandated group and feels challenged by one member's frequent objections. The counselor has begun responding with sarcasm and reminding the member that attendance is not voluntary.

What should the counselor do?

A) Recognize the misuse of positional power, stop the sarcastic responses, and use supervision to restore respectful and consistent facilitation

B) Continue emphasizing the mandate because authority is the most effective response to group resistance

C) Remove the member from the group because objections show unwillingness to benefit from counseling

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Question 22

> An evening parenting program has high dropout among single parents. Participants report that sessions overlap with shift work and that no child care is available. Staff members have been describing the departures as lack of motivation.

Which response best combines staff development and advocacy?

A) Train staff to explain the attendance expectations more firmly before enrollment

B) Learn about the practical barriers with participants and advocate for scheduling, child-care, or delivery options that address them

C) Limit future enrollment to parents who can guarantee attendance at the current time

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Question 23

> An advertisement promises a "certified trauma expert" title after a six-hour self-paced course. It gives no information about instructor qualifications, skill assessment, supervised practice, or recognition by a professional body.

How should a counselor evaluate this opportunity?

A) Compare the course with credible competency standards, evidence, instructor qualifications, and opportunities to demonstrate skill

B) Enroll because use of the word certified means the title is recognized across counseling settings

C) Enroll if the counselor already treats trauma because experience will supply all missing training components

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Question 24

A counselor in stable recovery from stimulant use begins treating a client who describes recent use in vivid detail. After several sessions, the counselor notices unexpected cravings and increasing difficulty concentrating during the client's appointments.

What should the counselor do?

A) Promptly use supervision and recovery supports to assess functioning and arrange transfer if competent care cannot be maintained

B) Disclose the counselor's recovery history so the client can avoid descriptions that trigger cravings

C) Continue without consultation because personal recovery should make the counselor more effective with substance-use concerns

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Question 25

A client repeatedly arrives late. The counselor, who grew up in a household where lateness was treated as disrespect, begins ending this client's sessions earlier than the agency attendance policy requires and feels satisfied when the client becomes upset.

What is the most appropriate response?

A) Keep the stricter consequence because the client's emotional reaction shows that the limit is effective

B) Examine the personal meaning of lateness, consult as needed, repair the inconsistency, and apply policy based on clinical facts

C) Avoid addressing attendance in the future because the counselor can no longer set an objective limit

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: C

Correct Answer: Bring the performance concern to the clinical supervisor and develop a monitored improvement plan, using consultation only as a supplement

Explanation: The omissions concern current clinical performance within an existing evaluative supervision relationship. The supervisor needs to help assess risk, observe improvement, and determine appropriate oversight; outside consultation may add expertise but cannot replace that responsibility.

Question 2 — Correct: A

Correct Answer: Obtain independent education on translation errors, cultural and linguistic bias, confidentiality, human review, and appropriate disclosure

Explanation: Machine-generated clinical translations can create meaning and access errors that fluency checks after use may not catch. Independent education prepares the clinic to evaluate limitations, review processes, privacy, and communication responsibilities before relying on the materials.

Question 3 — Correct: B

Correct Answer: Examine the attraction to the role, obtain consultation, and decline the overlapping position while counseling is active

Explanation: The board role would place the counselor in decisions involving the active client, and the counselor's attraction to prestige may minimize that risk. Consultation and separation of the roles protect objectivity and the client's interests more effectively than limited abstention.

Question 4 — Correct: B

Correct Answer: Reflective practice examines experience in a structured way to inform future action, whereas rumination repeats distress without useful movement

Explanation: Reflection links examination of experience to learning and a realistic next step. Rumination tends to recycle distress without generating new understanding or action, and the distinction does not depend on whether the process occurs alone.

Question 5 — Correct: C

Correct Answer: Locate a supported version or consult qualified assessment and cultural-language experts before using translated scores

Explanation: Clinical fluency does not establish competence in test translation or preserve validity through literal wording changes. A supported version or expert-guided process is needed before the scores can be interpreted as measuring the intended construct.

Question 6 — Correct: B

Correct Answer: Engage affected people, define the barrier with relevant data, and identify feasible targets and partners for change

Explanation: Effective advocacy is grounded in the experiences of those affected and a clear understanding of the barrier and decision context. Premature solutions or public criticism can misidentify the problem and exclude useful partners.

Question 7 — Correct: A

Correct Answer: Mentoring can support career growth and professional identity but does not replace required clinical supervision or establish competence by itself

Explanation: Mentors may provide perspective, encouragement, and career guidance without holding the formal oversight responsibilities of a clinical supervisor. Discussion with a mentor does not transfer accountability or independently demonstrate clinical competence.

Question 8 — Correct: B

Correct Answer: Use role standards, local outcome data, and stakeholder input to propose duties aligned with students' counseling needs

Explanation: Role standards and local evidence create a professionally grounded case for preserving counseling and prevention functions. Collaborative advocacy is more likely to improve the plan than unilateral refusal, while passive acceptance would disregard the documented student impact.

Question 9 — Correct: C

Correct Answer: Promptly assess fitness with appropriate medical and supervisory support and arrange coverage or workload changes when needed

Explanation: Slowed thinking and loss of the clinical narrative may already affect competent performance. The counselor should obtain appropriate assessment and oversight while protecting upcoming clients, rather than proceeding automatically or asking clients to evaluate impairment.

Question 10 — Correct: C

Correct Answer: Review current authoritative standards and guidance, use case-based education, and seek consultation on emerging questions

Explanation: Ethical knowledge changes and must be interpreted in new practice contexts. Current authoritative materials, applied education, and consultation provide a stronger learning process than relying on graduate training or informal local custom alone.

Question 11 — Correct: A

Correct Answer: Examine the adaptation process, evidence, population limits, and relevant community perspectives before generalizing its use

Explanation: The phrase culturally adapted does not demonstrate how an intervention was developed, for whom it is supported, or whether affected communities informed it. Critical appraisal avoids both uncritical adoption and blanket rejection of culturally responsive adaptations.

Question 12 — Correct: C

Correct Answer: Examine the conflict in consultation, evaluate ongoing clinical need, and make transition decisions based on client welfare

Explanation: The counselor's financial interest may be affecting termination decisions even though goals have been met. Consultation and a fresh evaluation of clinical need help separate business pressure from client welfare and informed transition planning.

Question 13 — Correct: A

Correct Answer: Assess current ability promptly and arrange appropriate coverage or rescheduling rather than beginning while significantly distracted

Explanation: The counselor's physical and cognitive response suggests a current inability to give a complex intake adequate attention. A brief fitness assessment and continuity plan protect the prospective client without requiring detailed personal disclosure.

Question 14 — Correct: C

Correct Answer: Explain the service limitation, coordinate connection with the mobile team, and provide an appropriate bridge if feasible

Explanation: The counselor's available modalities do not match the client's current ability to access care, while a fitting service has openings. Transparent coordination and a bounded bridge support continuity better than making office attendance or unreliable public internet conditions of treatment.

Question 15 — Correct: A

Correct Answer: Evaluate the impact with supervision, use appropriate personal support, and consider rotating duties or reducing cumulative grief exposure

Explanation: Avoidance, guilt about positive experiences, and reduced emotional presence suggest cumulative grief exposure is affecting personal and professional functioning. A layered response can address the impact while preserving client care, whereas total social avoidance or permanent departure is premature.

Question 16 — Correct: B

Correct Answer: Explain the environmental limitation and facilitate care with the available qualified counselor

Explanation: The counselor cannot safely provide sessions in the available space, and a qualified accessible alternative is immediately available. Asking the client to forgo a required service animal or waiting for an allergic event would place the burden and risk on the client.

Question 17 — Correct: C

Correct Answer: Learn about economic access barriers and use client data to advocate for feasible scheduling or service alternatives

Explanation: The discontinuation pattern is linked to unpaid leave and limited hours, not simply individual time management. Combining contextual learning with client data supports targeted changes rather than equating identical schedules with equitable access.

Question 18 — Correct: C

Correct Answer: Explain the instrument's limits, advocate for appropriate interpretation, and seek assessment consultation if needed

Explanation: Competence includes recognizing limits on how assessment results may be used, not only administering the tool correctly. A brief screener should inform broader evaluation rather than become an automatic discharge rule, and altering scores would invalidate the process.

Question 19 — Correct: A

Correct Answer: Seek supervision promptly, reinforce professional boundaries, and arrange a careful transfer if objectivity cannot be maintained

Explanation: Changes in dress and unnecessary message review show that the attraction is beginning to influence professional behavior. Early supervision and firm boundaries may protect care, with transfer considered if the counselor cannot maintain objectivity; disclosing the attraction would burden the client.

Question 20 — Correct: B

Correct Answer: Learn from qualified community and professional sources, examine the counselor's assumptions, and help the agency design feasible access options

Explanation: The attendance pattern has credible structural explanations that the counselor initially misread as low commitment. Community-informed learning and service adaptation address both the assumption and the practical barriers without treating occupational similarity as a prerequisite for competence.

Question 21 — Correct: A

Correct Answer: Recognize the misuse of positional power, stop the sarcastic responses, and use supervision to restore respectful and consistent facilitation

Explanation: Mandated participation increases the importance of using authority carefully rather than humiliating a member for dissent. The counselor must address the personal reaction and facilitation behavior; coercion or removal based only on objections would intensify the power problem.

Question 22 — Correct: B

Correct Answer: Learn about the practical barriers with participants and advocate for scheduling, child-care, or delivery options that address them

Explanation: Participant reports identify concrete access barriers rather than lack of interest. Staff learning and feasible program changes respond to those conditions, while stricter messaging or exclusion would preserve the barrier.

Question 23 — Correct: A

Correct Answer: Compare the course with credible competency standards, evidence, instructor qualifications, and opportunities to demonstrate skill

Explanation: A certificate label does not establish that training meets meaningful professional standards. Evaluating instruction, evidence, assessed practice, and recognition helps the counselor distinguish a marketing title from genuine competence development.

Question 24 — Correct: A

Correct Answer: Promptly use supervision and recovery supports to assess functioning and arrange transfer if competent care cannot be maintained

Explanation: The emerging cravings and concentration difficulty require prompt evaluation of both counselor wellness and client care. Supervision and recovery supports may permit safe continuation, but transfer is appropriate if functioning cannot be stabilized; client disclosure would shift responsibility inappropriately.

Question 25 — Correct: B

Correct Answer: Examine the personal meaning of lateness, consult as needed, repair the inconsistency, and apply policy based on clinical facts

Explanation: The counselor's history and satisfaction at the client's distress show that a personal reaction is shaping a harsher limit. Reflection, repair, and consistent application restore fairness while still allowing attendance to be addressed clinically.
